



Pressing need for clinical trial research on dimensional personality disorder

Published Online

November 21, 2024

[https://doi.org/10.1016/S2215-0366\(24\)00365-1](https://doi.org/10.1016/S2215-0366(24)00365-1)

This online publication has been corrected. The corrected version first appeared at [thelancet.com/psychiatry](https://www.thelancet.com/psychiatry) on December 4, 2024

A decade has passed since the introduction of dimensional diagnostic systems for personality disorder.¹ However, national health guidelines (such as those from the UK's National Institute of Clinical Excellence) have not been updated to reflect changes from the previous categorical model of personality disorders.

We conducted a scoping review to examine whether there is enough clinical evidence to warrant the development of national guidelines for dimensional personality disorder. We focused on high-quality evidence from randomised controlled trials (RCTs) on the treatment of dimensional personality disorder, and particularly its recent criterion of severity. We conducted our literature searches from 2013 (introduction of dimensional personality disorder in DSM-5) to Oct 24, 2024, using search terms informed by the latest Cochrane reviews on personality disorder (appendix p 6).

After removing duplicates, we identified 3320 records, of which 862 were considered relevant. Nearly all (99%) relevant reports were based on categorical personality disorders. Most of these (n=517, 60%) focused on borderline personality disorder (BPD), with over half of these (n=320, 62%) being RCTs. The second most researched personality disorder was antisocial personality disorder, with 84 reports, of which 28 (33%) were RCTs. There were only a few RCTs on other personality disorders, including five on avoidant personality disorder, four on narcissistic personality disorder, two on schizotypal personality disorder, and none on the obsessive-compulsive, dependent, schizoid, histrionic, and paranoid personality disorders.

Since 2013, we have identified only three clinical trials on dimensional personality disorder. One of these trials examined a low-intensity intervention for personality disorder, showing promising results in the reduction of social dysfunction but warranting replication due to its pilot nature.² A second study examined the use of personality disorder severity in predicting treatment discontinuation, revealing that among inpatients with personality disorder, those with the lowest levels of self-functioning were the most likely to discontinue therapy.³ A final study conducted secondary longitudinal analyses on a six-year RCT of short-term hospital-based psychotherapy for personality disorder, showing that increases in dimensional personality constructs (eg, relational skills) predicted improvements in global functioning.⁴

The identification of only three trials on dimensional personality disorder points to a concerning lack of RCT-based evidence on the clinical management of this disorder. This absence of RCT research stands in stark contrast to the extensive body of empirical research validating the dimensional conception of personality disorder, including its internal consistency, inter-rater reliability, construct validity, and acceptability by clinicians and patients alike.¹ Recent work also supports the predictive superiority of dimensional personality disorder over categorical personality disorders by revealing that dimensional personality difficulties (such as personality disorder severity and traits) are stronger predictors of various clinical outcomes (including general psychiatric severity, psychosocial functioning, and quality of life), compared to categorical personality diagnoses.¹

Despite these empirical advances, it appears that clinical trial research has not progressed in parallel to investigate how these personality difficulties ameliorate within RCTs. There is a pressing need to conduct

such RCTs, focusing on the treatment of dimensional personality difficulties (including severity, suicidality, and social functioning) as well as whether that treatment is influenced by relevant comorbidities (such as autism and post-traumatic stress).⁵ Only with such rigorous clinical trial research can evidence-based guidelines be created for the management of this serious class of mental health problems.

SP is supported by the University College London Hospitals NIHR Biomedical Research Centre. PF is a Senior Clinical Advisor of the NHS CYP Mental Health Programme. All other authors declare no competing interests.

*Orestis Zavlis, Patrick Luyten, Steve Pilling, Peter Fonagy
orestis.zavlis.23@ucl.ac.uk

Research Department of Clinical, Educational, and Health Psychology, University College London, London WC1E 6BT, UK (OZ, PL, SP, PF); Faculty of Psychology and Educational Sciences, KU Leuven, Leuven, Belgium (PL)

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Coercive hospitalisation in China

The relationship between individual autonomy in decision making and coercive measures is particularly complex in the field of mental health, especially for people with severe mental illness. In *The Lancet Psychiatry*, Astrid Moell and colleagues¹

See Online for appendix